**CHIEF COMPLAINT:**

Depressed mood, pervasive anhedonia, severe early-morning awakening, and psychomotor slowing over the past four months.

**HISTORY OF PRESENT ILLNESS:**

The patient is a 42-year-old female who presents for comprehensive psychiatric evaluation. She reports a progressive inability to experience pleasure in activities she previously enjoyed. She describes waking at 03:30 AM daily with racing negative thoughts and inability to return to sleep. Appetite has decreased, resulting in an intentional 5 kg weight loss over two months. Energy levels are severely depleted.

**MENTAL STATUS EXAMINATION:**

The patient appears disheveled with slumped posture. Eye contact is poor and intermittent. Speech is slow, soft, and latencies are prolonged. Mood is described as 'empty and exhausted'; affect is constricted and tearful. Thought process is linear and goal-directed without loose associations. Thought content reveals prominent feelings of excessive guilt, worthlessness, and helplessness. No auditory or visual hallucinations. Denies active suicidal ideation, intent, or plan. Cognition is intact; insight and judgment are preserved.

**DIAGNOSIS AND TREATMENT:**

1. Major Depressive Disorder, single episode, severe, without psychotic features.
2. Rule out secondary medical etiologies: TSH, free T4, vitamin B12, and complete blood count ordered.
3. Initiate oral sertraline hydrochloride 50 mg daily, with planned titration to 100 mg daily in two weeks.
4. Referral for outpatient cognitive behavioral therapy (CBT).
5. Crisis hotlines and safety plan reviewed and provided.